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GUIDELINES FOR SUBMISSION OF PLASMA MASTER FILE

These guidelines provide recommendations to manufacturers who wish to submit Plasma Master Files in support of the quality of the starting materials for Plasma-Derived Medicinal Products (PDMP). These apply to applications for the registration or variations of PDMP. These guidelines emphasize South African Health Products Regulatory Authority's (SAHPRA) commitment towards the safety, efficacy, and quality of medicines. SAHPRA reserves the right to request any additional information to establish the safety, efficacy, and quality of a medicine in keeping with the latest knowledge at the time of evaluation. Alternative approaches may be used but the knowledge that would have been utilized to inform a decision should be scientifically and technically justified. SAHPRA is committed to ensure that all registered medicines will be of the required safety, efficacy, and quality. It is important that applicants adhere to the administrative requirements to avoid delays in the processing and evaluation of applications.

These guidelines and application forms are also available on www.sahpra.org.za

Document History

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Glossary

Abbreviation/ Term	Meaning
B19	Parvovirus B19
BWP	Biologics Working Party
CE	Conformité Européenne
CHMP	Committee for Medicinal Products for Human Use
DNA	Deoxyribonucleic acid
EC	European Committee
EMA	European Medicines Agency
EMEA	Europe, the Middle East and Africa
EU	European Union
FTT	First time tested
GMP	Good Manufacturing Practice
HAV	Hepatitis A virus
HBsAg	Hepatitis B surface antigen
HBV	Hepatitis B virus
HCV	Hepatitis C virus
NAT	Nucleic acid testing
(PDMPs)	Plasma-derived medicinal products
PMF	Plasma Master File

Abbreviation/ Term	Meaning
RT	Repeat tested
SAHPRA	South African Health Product Regulatory Authority
TRS	Technical Report Series
UK	United Kingdom
USA	United States of America
WHO	World Health Organization

Definitions

Definition	Meaning
Blood establishment,	Any structure, facility or body that is responsible for any aspect of the collection, testing, processing, storage, release and/or distribution of human blood or blood components when intended for transfusion or further manufacturing.
Blood collection site	Blood establishments where whole blood is collected from which plasma (recovered plasma) is separated. This is generally a public not-for profit organisation licensed and inspected by the Department of Health or, Plasma collection centres for source plasma which may be public/not for profit centres or private/commercial centres
CE-marked test	CE (Conformité Européenne) certification is an EU safety directive that indicates that a product or device has passed certain tests and means that a product can legally be sold anywhere within the EU and the European Economic Area.
First time donor	Someone who has never donated either blood or plasma.
First time tested donor	A person tested for the first time for infectious disease markers (with or without donation) and without evidence of prior testing in a given blood system.
Incidence	Rate of newly acquired infection identified over a specified time period.
Look-back procedure	A multistep process a blood establishment must complete, as a response to a donor who is newly reactive for an infectious disease test.

Definition	Meaning
Marketing authorisation	Is issued from a therapy regulatory agency that allows a therapy to be placed on the market, i.e. to be manufactured and sold outside research setting in a defined population.
Plasma-derived medicinal products (PDMPs)	Pharmaceuticals made from human plasma, the liquid part of blood.
Mobile blood donation centre	A vehicle or stand-alone caravan where blood is collected from donors. These vehicles are equipped with everything needed to perform blood donations.
Prevalence	In epidemiology, it measures existing cases within a defined group, typically expressed as a percentage, fraction, or rate per 10 000 or 100 000 population.

Recovered plasma	Plasma collected through whole blood donation in which plasma is separated from its cellular components. Recovered plasma may be used for fractionation.
Regular donor	Someone who routinely donates blood or plasma (i.e. within the last two years), in accordance with minimum time intervals, in the same donation centre.
Repeat tested donor	Person whose blood/plasma has been tested previously for infectious disease markers in a given blood system.
Source plasma	Plasma collected through apheresis, a process that takes only plasma from the donor while the cellular components are returned to the donor.
(v)CJD: Variant Creutzfeldt-Jakob disease	Is a rare disease caused by a prion, an infectious protein. It is linked to eating beef from cows infected with "mad cow disease." It is always fatal, usually within 14 months of the first symptoms.
Voluntary non-remunerated blood donor	Someone who donates blood without receiving payment. They donate blood of their own free will and may donate plasma or other blood components.

1. INTRODUCTION AND PRINCIPLES OF A PMF

The starting material of plasma-derived medicinal blood products should comply with Ph. Eur. and cGMP (Directive 2001/83/EC EU), *WHO Requirements for the collection, processing and quality control of blood, blood components and plasma derivatives, Annex 2, TRS No 840*, and *WHO recommendations for the production, control and regulation of human plasma for fractionation, Annex 4, TRS No 941*).

Note: the applicant for the PMF may decide to comply with either the EU or WHO directives.

Both have similar standards and are acceptable. Suppliers in Europe do not in all cases comply solely with EU requirements.

The SAHPRA Guidelines on the Requirements of a Plasma Master File (PMF) lay down the specific requirements for the plasma, plasma pools and their constituents and excipients, which are part of the starting material of the medicinal products for which the PMF is applicable. This information applies to the selection of donors, and the collection and testing of the blood and blood components. In addition, every blood establishment or plasma collection centre must be approved and inspected annually according to GMP by a Competent Authority. Approval is required for processing, storage and transport of blood and blood components, including plasma, intended for the manufacturing of medicinal products.

Cryoprecipitate or other intermediates are not part of the PMF certification. The manufacturing process of the medicinal products, starting from the pooled plasma is not part of the PMF dossier. The PMF is a stand-alone document which is separate from the dossier for marketing authorisation. The dossier for marketing authorisation for each individual medicinal product should provide the information on the requirements of the human plasma that is used as starting material. Applicants that use the PMF certification system must identify and make reference to the PMF(s) in the dossier of each medicinal product.

The characteristics of plasma collected in countries or various regions of countries may differ. Such differences may be related to the prevalence or incidence of transfusion-transmissible diseases in the country or regions within the respective countries. The applicant should identify such differences and relate these to the possible increased risk posed by using plasma from such countries or regions.

Annual update

The PMF information shall be updated and re-submitted for approval on an annual basis. This annual update should include all changes and updated information since the last approval (initial or annual). The update should include a list for which it was retrospectively found there were indications that a donation contributing to a plasma pool was infected with HIV or hepatitis A, B or C or another transfusion-transmissible agent. It should also include the number of positive donations that have been identified per

viral marker by NAT testing at the fractionator level including minipool testing. If NAT testing of minipools is performed by the PMF holder, the number of positive donations should be given.

The plasma master file is a stand-alone document and describes the following:

- a) Plasma source and collection
- b) Epidemiological data on blood transmissible infections
- c) Characteristics of donations and selection/exclusion criteria
- d) Testing of blood/plasma donations and pools for infectious agents
- e) Plasma quality and safety
- f) Conditions of storage and transport of plasma; and
- g) A copy of the plasma specification and plasma pool batch analysis data.

1.1 Purpose

Applications for human plasma-derived medicinal products shall be evaluated based on the products' quality, safety and efficacy. This guideline outlines the requirements for the Plasma Master File (PMF) and the specific documentation required to support registration of these products.

1.2 Scope

These guidelines include submissions of Plasma Master files, whereby the plasma is used as a component or intermediates derived for producing plasma-derived medicinal products (PDMPs). The PMF guidelines will apply exclusively to Category A medicines manufactured using plasma or intermediates of plasma. These guidelines do not refer to Plasma Master Files that have valid certification of the PMF as per the European Commission

2. LEGAL PROVISION

The legal provision for the registration of medicines, medical devices, including In Vitro Diagnostics (IVDs), is included in the Medicines and Related Substances Act No. 101 of 1965 as amended and the gazetted General Regulations.

Specific reference is made to Section 15(1) - (10) of the Act and Section 16(1) - (10) of General Regulations.

3. GENERAL INFORMATION

3.1 Plasma-derived products' list

The plasma - derived products list provides an annex plasma-derived medicinal products which are relevant to the PMF. In addition, it includes investigational products, cryoprecipitate and intermediates derived from the plasma of the PMF. And it states whether each of the listed medicinal products has been granted marketing authorisation or is in the process of being granted such authorisation by the relevant Competent Regulatory Authority(ies). Lastly, it includes the SAHPRA product registration number where applicable.

3.2 Overview of safety strategy

A critical overview of the overall strategy to ensure the safety of the plasma included in the PMF should be provided. This evaluation should include for each collection site the epidemiological data on blood transmissible infections known for the donor population, viral load limits for plasma pools and normal size of plasma pool., The system of donor selection criteria, , criteria for the use of recovered plasma (from whole blood donation) and/or source plasma (from plasmapheresis), measures that reduce the risk of (v)CJD, screening of donations, minipool strategy if relevant, the testing of the plasma pools, inventory hold, and “look-back” procedures should be described.

The information on centres where blood or plasma is collected, including mobile clinics, must be summarised. The applicant must also provide information on the test laboratories, the in vitro diagnostic devices used, and the sensitivity of the assays performed.

The estimated residual risk of undetected viraemic donations that may enter the production pool should be described and discussed (*WHO Guidelines on estimation of residual risk of HIV, HBV or HCV infections via cellular blood components and plasma, Annex 4, TRS No 1004*). It should be demonstrated how the facility's safety strategy ensures that the measures taken throughout the collection, processing, testing, storage and transport of the plasma robustly ensures that the plasma pool is safe.

3.2.1 General logistics

A flow-chart, describing the complete supply-chain for plasma from collection to the manufacturing process of the plasma pool, must be provided. This should include all establishments concerned in the collection, testing, processing, storage and transport of blood or plasma and their relationship. The transport chain should include details of international transport and customs.

4. INFORMATION ON STARTING MATERIALS

The quality and safety of plasma-derived medicinal products depend on the source of the plasma and the manufacturing process of the medicinal product. The collection, testing, processing, storage and transport of human plasma are thus major factors in the quality assurance of the manufacturing process of plasma-derived medicinal products. Blood establishments and centres should fulfil the requirements, be inspected and approved according to the relevant directives of the Competent Regulatory Authority.

4.1 Plasma origin

4.1.1 Information on centres or establishments in which blood/plasma collection is carried out, including inspection and approval, and epidemiological data on blood transmissible infections. Provide, in tabular format, a complete list of names and addresses of blood establishments and centres from which plasma is collected. If mobile or temporary equipped centres are used, the relationship with their respective blood establishments should be provided. There should be a statement that these mobile and temporary centres operate under the same quality system as the blood establishment that they relate to.

It provides information about internal audits (frequency, date of last audit and outcome of last inspection); and audits by regulatory authority (frequency, date of last audit and final outcome of last inspection).

The suppliers of plasma, for which special criteria are applicable (such as anti-D), should be identified.

The collection and processing operations of the blood establishments and centres should be described and summarised. If establishments or centres are no longer used or are temporarily closed, they should be listed in a stand-alone table indicating the date when use of the centre or establishment has ceased and the reason.

If there is a separate look back department, the address, duties, and approval status by a recognised Competent Authority should be given.

4.1.2 Characteristics of donations

Specify whether donors are non-remunerated or remunerated. Justify why donors are regarded as voluntary and non-remunerated. If applicable, the nature of any compensation for donation should be given.

4.1.3 Epidemiological data on blood transmissible infections

Should be submitted in accordance with the EMA *Guideline on Epidemiological Data on Blood Transmissible Infections* (EMA/CHMP/BWP/548524/2008)

4.2 Information on centres or establishments in which testing of donations and plasma pools are carried out, including inspection and approval status.

Specify the test laboratory used by the establishment or centre. If test laboratories are no longer used, or are temporarily not used, they should be listed in a stand-alone table indicating the date when use of the laboratory ceased and the reason.

4.3 Selection or exclusion criteria for blood or plasma donors

The criteria confirm that each establishment or centre complies with the accepted selection or exclusion criteria for blood or plasma donors as per CHMP guidance documents, WHO, and Council of Europe recommendations. It is recognised that there may be some challenges with the EU donor selection criteria, especially concerning tropical diseases that lead to temporal deferral in Europe.

Donor selection criteria must be accompanied by a questionnaire that is specific for the countries included. Specify any requirements versus emerging infectious agents in a specific country of collection and confirm that selection/exclusion criteria for blood/plasma donors complies with such requirements. This should include the data of the prevalence/incidence of infectious agents that are present in the country but are not screened for by the testing protocol.

If the epidemiological data of the proposed selection or exclusion criteria suggest that the plasma may pose a higher risk of transmitting infectious disease than of plasma collected in developed countries such as EU, USA, UK, Australia and Canada, this should be discussed and the selection criteria for donors justified. The upper limits for acceptance of donors based on the prevalence/incidence of transmissible diseases should be set and justified. This should, where applicable, be discussed for the country, regions within the country and for individual collection centres.

Indicate whether the plasma from the proposed PMF collection site will be pooled with plasma procured from other establishments or whether it will be kept separately. Indicate whether the plasma-derived medicinal products will be distributed to all patients served by the manufacturer or only to patients in the country of origin of the plasma of the PMF.

The exclusion criteria for (v)Creutzfeldt-Jakob disease should be stated and comply with EMEA and CHMP guidance.

4.4 System in place which enables the path taken by each donation to be traced from the blood or plasma collection establishment through to finished products and vice versa.

It summarises the implemented system that enables the path of each donation to be traced from the Blood or plasma collection facility through to the finished product. This must include the testing facility, identification procedures, labelling and record keeping.

Give information on steps that would be taken if it was found retrospectively that donation(s) should have been excluded from processing (“look-back procedure”) and justify the system in accordance with accepted guidelines and procedures.

4.4.1 Look-back procedure

If a plasma unit fails release due to any of the reasons listed below, a look-back should be instituted. The process consists of tracing back and the additional testing of previous donations for at least six months prior to the last negative donation. The following procedures should be documented in a standard operating procedure:

- a) it is found that the donor did not meet the relevant donor health criteria
- b) a subsequent donation from a donor previously found negative for viral markers is found positive for any of the viral ~~markers~~
- c) it is discovered that testing for viral markers has not been carried out according to agreed procedures
- d) the donor has developed an infectious disease caused by an agent potentially transmissible by plasma-derived products (HBV, HCV, HAV and other non-A, non-B, non-C hepatitis viruses, HIV 1 and 2 and other agents in the light of current knowledge)
- e) the donor develops Creutzfeldt-Jakob disease
- f) the recipient of blood or a blood component develops post- transfusion/infusion infection which implicates or can be traced back to the donor

5. PLASMA QUALITY AND SAFETY

5.1 Confirm that the plasma quality complies with the *Ph Eur Monograph for Human Plasma for Fractionation (Monograph 0853)* or an equivalent standard such as the *WHO Recommendations for the Production, Control and Regulation of Human Plasma for Fractionation*.

5.2 Testing of blood/plasma donations and pools for infectious agents, including information on test methods and, in the case of plasma pools, validation data on the tests used.

Provide information on

- Screening tests for markers of infection as per Ph Eur or WHO requirements
- Any other screening tests carried out.
- List of tests performed on individual donations, including NAT testing and CE mark if applicable;
- Validation of these screening procedure methods; and
- Provide details of procedures for any inventory hold and quarantine procedure for plasma. The inventory hold requirement should be based on scientific evidence such as test kit sensitivity and should align with the EMA requirements of an overall safety strategy.
- When minipools are tested details must be provided and the use of the process justified.
- Provide criteria for acceptance or rejection of donation/pool and whether there is a re-testing policy

Test	Test Performed on:		
	Individual Donation	Minipool (size) (if relevant)	Plasma Pool
HBsAg			
HIV 1 and 2 Antibody			
HCV Antibody			
HCV RNA			
B19 DNA			
Other tests			

5.3 Validation of testing methods

Testing of donations

- **Serology markers**
Tests must be carried out in accordance with manufacturer's directions. Tests should be state-of-the-art in terms of seroconversion sensitivity.
- For CE-marked test kits submission of validation data is not required. For non-CE-marked test kits, the applicant should demonstrate that they comply with equivalent standards
- **NAT**
Describe the analytical procedures of mini pool testing as part of the screening of individual donation. Summarise the validation of the test system and provide reports indicating specificity and the detection limits.

Viral marker testing of the plasma pools provides a description of each test method and the respective validation report. The sensitivity of the test for each marker as a function of pool size must be included.

- **NAT testing of the plasma pools**

Should comply with the requirements of a recognised authority: EMA *Guideline on Validation of Immunoassay for the Detection of Antibody to human Immunodeficiency Virus (Anti-HIV) in Plasma Pools*,

If mini pool testing by NAT is part of the screening of individual donations, a brief description of the analytical procedures for NAT methods should be provided if non-CE-marked tests are used (in-house methods or commercial kits). Validation is not required for mini pools testing by NAT if the test is CE marked for this purpose. However, information on the detection limit, as related to the single donation, should be provided.

For every laboratory that carries out plasma pool testing for viral markers, provide a description of each test method and the respective validation report.

Information on the sensitivity of the test for each marker as a function of pool size should be included. All NAT methods used for plasma pool testing should comply with the requirements in Ph Eur *General Methods 2.6.21 Nucleic acid amplification techniques*. For every laboratory that carries out plasma pool testing by NAT, provide a description of each NAT test method and the respective validation report. As recommended in this guideline, the ability of the analytical procedure to detect all HCV genotypes should be demonstrated.

- **Proficiency studies**

Recommended. Provide information and reports.

5.4 Technical characteristics of bags for blood and plasma collection including information on anticoagulants

Provide information on quality and identity of plastic material, leachables, sterilization procedures, composition, results of a real time storage stability study of plasma in the container. Due to receiving blood products from other countries that are not aligned with EMA, CE-marked bags may be accepted and be tested accordingly.

Document verification that the source materials do not contain any additives other than citrate or acid citrate dextrose anticoagulant solution, unless it has been shown that the processing method yields a final product free of the additive to such an extent that the continued safety, purity, potency, and effectiveness of the final product is not adversely affected.

5.5 Conditions for processing, including freezing and storage of plasma for every collection and processing center.

They describe the conditions of storage of plasma for every establishment responsible for storage. Confirm compliance with Ph Eur, WHO or other internationally accepted requirements.

Also, they describe the conditions of storage including transport flow from collection centers to interim and final storage sites.

Lastly, they list the organisations involved in transport and summarise the system in place that ensures that transport is performed under appropriate conditions and complies with GMP. Indicate date and outcome of last inspection by a Competent Regulatory Authority.

5.5.1 Procedures for any inventory hold period

Provide details and justification of any inventory hold and quarantine procedure for plasma.

5.5.2 Characterisation of plasma pool

Provide addresses of sites where plasma is pooled.

Provide information on:

5.5.2.1 Plasma pool preparation. Describe procedures such as the thawing process and visual inspection of containers before pooling, Indicate the size of the plasma pool in number of donations and in litres.

5.5.2.2 Sampling of plasma pool. Define the pool from which the samples for viral marker testing are obtained. Describe the sampling procedure.

5.6 Agreement in place between the plasma-derived medicinal product manufacturer and/or plasma fractionator/processor and blood establishments which defines the conditions of their interaction and responsibilities and the specifications of the plasma.

The agreement confirms that a contract exists between the blood establishments and the manufacturer and/or the PMF holder to ensure the interaction between them, and the fulfilment

of the relevant Good Manufacturing Practice (GMP) or Good Practice requirements.

In the event of a serious failure of a center being discovered, the manufacturer and SAHPRA will be immediately informed.

6. EPIDEMIOLOGICAL DATA

The data should be reported and evaluated as per the EMA *Guideline on the scientific data requirements for a plasma master file*.

The systems to monitor changes of epidemiological data must be described. The information should be separate for repeat tested (RT) and first-time tested (FTT) donors of source and recovered plasma and given for each blood establishment/blood or plasma collection center and country as appropriate.

There should be an overview of how the epidemiological data of the individual collection centers are compared. The in-house alert limits to identify outliers should be given for the FTT and RT donors for source and recovered plasma and for donors from other countries. The in-house alert limits should be justified. The corrective actions taken if a collection center exceeds alert limits must be given and justified.

If the epidemiological data of the PMF suggest that the plasma may pose a higher risk of transmitting infectious disease than plasma collected in developed countries such as the EU, USA, UK, Australia and Canada, this should be discussed and the use of the plasma of the PMF justified. Plasma with unjustifiable higher risk of transmitting infectious diseases must not be used for the manufacture of PDMPs.

6.1 Selection of donors

The donor selection and exclusion criteria should be provided and comply with international accepted standards. Donors are voluntary and non-remunerated. (*Directive 2002/98/EC of the European Parliament and of the Council of 27 January 2003: Setting standards of quality and safety for the collection, testing, processing, storage and distribution of human blood and blood components*); WHO *Blood donor selection: guidelines on assessing donor suitability for blood donation*.

6.2 Testing of donations

The testing of donations should comply with international directives (*Directive 2002/98/EC of the European Parliament and of the Council of 27 January 2003: Setting standards of quality and safety for the collection, testing, processing, storage and distribution of human blood and blood components*; WHO *recommendations for the production, control and regulation of human plasma for fractionation, Annex 4, TRS No 941*).

It is noted that due to the upcoming European Union's Regulation on Standards of Quality and Safety for Substances of Human Origin (SoHO) Regulation, the Blood directive and corresponding technical directives will cease to exist after July 2027 and will be replaced by guidance of the EDQM (European Directorate for the Quality of Medicines & HealthCare, a directorate of the Council of Europe).

6.3 Testing of plasma pools and donations.

NAT Testing of individual donations or testing of minipools should be described and pool sizes justified. The tests should be for Hepatitis B DNA, Hepatitis C RNA, HIV-1 RNA and HIV-2 RNA.

6.4 Inventory hold and post-collection measures including look back procedure

Demonstrate and discuss traceability of a donation from the site where the blood /plasma is collected through to the finished product and vice versa. Traceability systems should be in accord with EU and WHO directives. The data and information on traceability for collection centres that have been closed or do not deliver plasma anymore should be kept for 30 years.

The PMF applicant must demonstrate that there is an inventory hold period of 45 days from the date of donation to the first step of the manufacturing process.

6.5 Residual risk estimation

The residual risk posed by donations that contain viruses not detected by the routine screening tests must be calculated in accord with internationally accepted methods such as WHO *Guidelines on estimation of residual risk of HIV, HBV or HCV infections via cellular blood components and plasma, Annex 4, TRS No 1004*.

7. REFERENCES

Only important relevant documents are referenced:

1. EMEA Guideline on requirements for Plasma Master file Certification (https://health.ec.europa.eu/document/download/b19375af-8a96-49b6-9faf-e773220fd1a9_en?filename=466303en_08_2004_en.pdf)
2. CHMP Guideline on the Scientific Data Requirements of a Plasma Master File (<https://www.ema.europa.eu/en/scientific-data-requirements-plasma-master-file-scientific-guideline>)
3. WHO Requirements for the collection, processing and quality control of blood, blood components and plasma derivatives, Annex 2, TRS No 840 (<https://www.who.int/publications/m/item/annex2-who-trs-840>)
4. PIC/S Good Practice Guidelines for blood establishments and hospital blood banks. (<https://www.gmp-compliance.org/guidelines/gmp-guideline/pic-s-guide-to-inspections-of-source-plasma-establishment-and-plasma-warehouses-pi-008-4>)
5. WHO Recommendations for the Production, Control and Regulation of Human Plasma for Fractionation (https://cdn.who.int/media/docs/default-source/biologicals/blood-products/ecbs-2005-annex-4-human-plasma-fractionation.pdf?sfvrsn=50177626_2)

6. *EU Guidance on good manufacturing practice and good distribution practice: Questions and Answers* (<https://www.ema.europa.eu/en/human-regulatory-overview/research-development/compliance-research-development/good-manufacturing-practice/guidance-good-manufacturing-practice-good-distribution-practice-questions-answers>).
7. *Directive 2002/98/EC of the European Parliament and of the Council of 27 January 2003: Setting standards of quality and safety for the collection, testing, processing, storage and distribution of human blood and blood components* (https://www.ema.europa.eu/en/documents/regulatory-procedural-guideline/directive-200298ec-european-parliament-and-council-27-january-2003-setting-standards-quality-and-safety-collection-testing-processing-storage-and-distribution-human-blood-and-blood-com_en.pdf)
8. WHO *Blood donor selection: guidelines on assessing donor suitability for blood donation* (<https://www.who.int/publications/i/item/9789241548519>)
9. EMEA/CHMP/BWP/125/04 *Guideline on Epidemiological Data on Blood Transmissible Infections* (https://www.ema.europa.eu/en/documents/scientific-guideline/concept-paper-revision-guideline-epidemiological-data-blood-transmissible-infections-revision-2_en.pdf)
10. WHO *recommendations for the production, control and regulation of human plasma for fractionation, Annex 4, TRS No 941* (<https://www.who.int/publications/m/item/annex4-ecbs-human-plasma-fractionation>)
11. WHO *Guidelines on estimation of residual risk of HIV, HBV or HCV infections via cellular blood components and plasma* (<https://www.who.int/publications/m/item/annex-4-trs-no-1004>)
12. Guideline on Epidemiological Data on Blood Transmissible Infections, (EMEA/CHMP/BWP/548524/2008)
13. *Guideline on Assessing the Risk for Virus Transmission – Chapter 6 of the Note for Guidance on Plasma-derived Medicinal Products* (CHMP/BWP/5180/03).
14. WHO *Recommendations for the Production, Control and Regulation of Human Plasma for Fractionation* (https://cdn.who.int/media/docs/default-source/biologicals/blood-products/ecbs-2005-annex-4-human-plasma-fractionation.pdf?sfvrsn=50177626_2)
15. *Guideline on Validation of Immunoassay for the Detection of Antibody to human Immunodeficiency Virus (Anti-HIV) in Plasma Pools*, (<https://www.ema.europa.eu/en/validation-immunoassay-detection-antibody-human-immunodeficiency-virus-plasma-pools-scientific-guideline>)

8. VALIDITY

This guideline is valid for a period of 5 years from the effective date of revision. It will be reviewed on this timeframe or as and when require.